

SpectrumReady Digital Guide

The Complete Therapy Roadmap

Evidence-Based Guide to Every Major Autism Therapy —
What Works, What Doesn't, and How to Build the Right
Team

\$47

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Important Notice — Please Read Before Proceeding

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Time-Sensitive Information. Government benefit amounts, income thresholds, and program eligibility rules change regularly. Dollar figures and thresholds in this guide reflect information as of 2025. Always verify current amounts and rules directly with the relevant federal or state agency before making any decisions.

Individual Circumstances Vary. Eligibility for the programs discussed depends on your individual circumstances. This guide describes how programs generally work — not whether you or your child specifically qualifies.

Consult Professionals. For decisions about your child's specific situation — including SSI applications, IEP disputes, insurance appeals, or estate planning — consult a qualified special education attorney, disability benefits counselor, or other licensed professional.

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Introduction: Building the Right Therapy Team

Your child's therapy plan will be the single largest investment of time and money your family makes over the next decade. Getting it right — knowing what to pursue, what to skip, and how to fund it — is what this guide is about.

The landscape of autism therapies is enormous and confusing. Some have decades of research behind them. Others are aggressively marketed with little or no evidence. A few are actively harmful. This guide cuts through the noise with an honest, evidence-based review of every major approach.

EVIDENCE RATING SYSTEM USED IN THIS GUIDE

★ = No credible evidence / anecdotal only

★★ = Preliminary / emerging evidence

★★★ = Moderate evidence, some methodological limitations

★★★★ = Strong evidence, well-replicated studies

★★★★★ = Extensive evidence, recognized by major medical/behavioral bodies

Part 1: How to Evaluate Any Therapy

The Four Questions to Ask Before Starting Any Intervention

1. **What does the research say?** Ask for peer-reviewed studies, not testimonials or theory.
2. **What specific outcomes are we targeting?** Every intervention should have measurable goals tied to your child's individual needs.
3. **How will we know if it's working?** Data collection, progress monitoring, and defined timeframes matter.
4. **What is the opportunity cost?** Every hour in one therapy is an hour not in another. Prioritize highest-evidence approaches first.

RED FLAGS FOR ANY THERAPY PROVIDER

- Claims their therapy "cures" autism
- Cannot explain the evidence base for their approach
- Discourages data collection or progress monitoring
- Discourages you from consulting other professionals
- Requires long-term prepaid contracts before demonstrating results

Part 2: Applied Behavior Analysis (ABA)

Evidence Rating: ★★★★★

ABA is the most researched behavioral intervention for autism. It is endorsed by the American Academy of Pediatrics, the US Surgeon General, and required to be covered by insurance in all 50 states. It uses principles of learning and motivation to teach skills and reduce barriers to learning.

What ABA Can Address

Domain	Examples
Communication	Requesting, labeling, conversation, AAC
Social skills	Joint attention, play, peer interaction, perspective-taking
Adaptive behavior	Toileting, dressing, grooming, mealtime
Reduction of barriers	Self-injurious behavior, aggression, elopement, severe tantrums
Academic readiness	Attending, instruction following, pre-academic skills

Types of ABA

- **Discrete Trial Training (DTT):** Structured, table-based teaching. Best for teaching new skills efficiently.

- **Natural Environment Teaching (NET):** Learning in natural contexts (play, daily routines). Promotes generalization.
- **Verbal Behavior (VB):** Language-focused ABA based on Skinner's analysis of verbal behavior.
- **Pivotal Response Treatment (PRT):** Child-led, naturalistic. Targets "pivotal" areas (motivation, self-initiation) that affect broader development.

Hours of Therapy — What the Research Says

Child Profile	Research-Supported Range
Younger children with significant needs (2–5)	25–40 hours/week (intensive)
School-age children with moderate needs	10–20 hours/week
Children with higher functioning profiles	5–15 hours/week (targeted)

Note: "More hours" is not always better. Quality, specificity of goals, and family involvement matter as much as quantity.

Questions to Ask an ABA Provider

- ☐ Is the supervising clinician a BCBA (Board Certified Behavior Analyst)?
- ☐ What is the ratio of BCBA supervision hours to RBT direct hours?
- ☐ How are goals developed — is it individualized to my child?
- ☐ How is data collected and shared with parents?
- ☐ What parent training do you provide?
- ☐ What is your approach to reducing challenging behaviors?

IMPORTANT: ABA QUALITY VARIES ENORMOUSLY

A good ABA program with experienced BCBAs and appropriate ratios produces excellent outcomes. A high-hours program with poor supervision and inadequate individualization does not. Credential the team, review the data, and trust your observations of your child's engagement.

Part 3: Speech-Language Therapy (SLT)

Evidence Rating: ★★★★★

Speech-Language Pathologists (SLPs) address communication across all modalities. For autism, SLT is almost always indicated and is one of the highest-impact investments you can make.

What SLT Addresses for Autism

Area	Description
Functional communication	Requesting needs, commenting, answering questions
Language development	Vocabulary, sentence structure, comprehension
Pragmatic/social language	Conversation, perspective-taking, understanding humor/sarcasm
Feeding and oral motor	Food selectivity, chewing, swallowing
AAC implementation	PECS, speech-generating devices, communication apps

AAC Is NOT a Last Resort

Research consistently shows that AAC (Augmentative and Alternative Communication) does not inhibit speech development — it supports it. For children who are non-speaking or minimally verbal, early AAC access is critical. Do not wait for speech to "see if it develops" before pursuing AAC.

Frequency Recommendations

Child Profile	Typical Recommendation
Minimally verbal / AAC user	3–5x/week with intensive parent training
Verbal but with significant pragmatic deficits	2–3x/week
Maintenance/generalization phase	1x/week with home practice

Part 4: Occupational Therapy (OT)

Evidence Rating: ★★★★★

OT addresses the skills needed for daily living and participation. For autism, OT commonly targets sensory processing, fine motor skills, self-care, and handwriting.

Sensory Processing — What OT Can Do

Many children with autism experience sensory differences — hyper or hypo sensitivity to touch, sound, movement, taste, or sight. OT using Sensory Integration Therapy works to help the brain better process and regulate sensory input.

Sensory Challenge	OT Strategies
Tactile hypersensitivity	Desensitization, deep pressure, weighted blankets
Auditory sensitivity	Noise management, auditory integration programs
Proprioceptive seeking	Heavy work activities, movement breaks
Vestibular sensitivity	Swinging, balance activities (graded)
Food texture sensitivity	Feeding therapy, systematic desensitization

Fine Motor and Daily Living Skills

- Handwriting (often significantly impacted in autism)
- Dressing, buttoning, zipping
- Scissor skills and tool use
- Feeding (fork, spoon, cup)
- Organizational skills and executive function strategies

Part 5: Additional Therapies — Evidence Review

DIR/Floortime — ★★★★★

Developmental, Individual Difference, Relationship-based model. Emphasizes following the child's lead, emotional engagement, and relationship-building. Good evidence for social-emotional development. Often used to complement ABA. Best with a trained DIR clinician plus significant parent training.

Social Skills Groups — ★★★★★

Structured groups targeting peer interaction, conversation, and social problem-solving. Most effective for verbal children in middle childhood and adolescence. PEERS (Program for the Education and Enrichment of Relational Skills) has the strongest evidence base.

Physical Therapy (PT) — ★★★★★ (for motor goals)

Many children with autism have co-occurring developmental coordination disorder. PT addresses gross motor development, balance, coordination, and functional mobility. Indicated when there are significant motor delays or coordination challenges.

RDI (Relationship Development Intervention) — ★★★★★

Parent-mediated approach focused on dynamic intelligence and relationship skills. Limited controlled research but conceptually aligned with current developmental approaches. Requires intensive parent training.

Music Therapy — ★★★★★

Evidence supports music therapy for social and communication outcomes. Particularly useful for children who respond strongly to music. Best as a complement to primary therapies, not a replacement.

Animal-Assisted Therapy — ★★★★★

Emerging evidence for social engagement, anxiety reduction, and motivation. Best as a supplement with a clear therapeutic goal. Costs not typically covered by insurance.

Therapies Lacking Evidence — Approach With Caution

Therapy	Evidence Status	Notes
Facilitated Communication	Discredited	AAP and ASHA advise against
Detoxification / chelation	No evidence, risky	Serious health risks documented
Hyperbaric oxygen	No reliable evidence	FDA warns against off-label use
Gluten-free / casein-free diet	Limited evidence	May help some with GI issues; consult pediatrician

Part 6: Finding and Vetting Providers

Where to Find Qualified Providers

- **BACB.com** — Verify BCBA credentials, find certified behavior analysts
- **ASHA ProFind** — Find certified speech-language pathologists
- **AOTA Practitioner Finder** — Occupational therapists
- **Autism Speaks Provider Directory** — Multi-specialty autism providers
- **Insurance provider directory** — In-network providers for billing
- **Parent recommendations** in local autism parent groups — often the best source

First Appointment Interview Guide

Use these questions for any new provider before committing:

1. What is your experience specifically with autism? How many clients with autism do you currently serve?
2. What certifications or specialized training do you have for autism?
3. How will you involve me as a parent in my child's treatment?
4. How will you measure and report progress?
5. What is your wait time and scheduling flexibility?
6. Do you accept our insurance? What is the billing process?
7. What happens if the current approach isn't working?

Part 7: Funding Your Therapy Plan

Funding Source Priority Order

Priority	Source	Why
1st	Private health insurance	Required by law to cover ABA in all 50 states
2nd	Medicaid / CHIP	EPSDT covers all medically necessary services
3rd	School district (IDEA)	Covers educationally necessary services at no cost
4th	Medicaid HCBS Waiver	Covers services not otherwise funded
5th	Private grants	Fill gaps in other coverage
6th	Out of pocket	Only after maximizing all other sources

Insurance Appeals for Therapy

The most common insurance denials for therapy:

- "Not medically necessary" — Counter with a Letter of Medical Necessity from the diagnosing physician and therapy provider
- "Experimental/investigational" — ABA is not experimental; cite ACA mandates and state law
- "Exceeded visit limits" — Request a medical exception; file an internal appeal, then external review

The complete insurance appeals process is covered in the companion guide "The Autism Insurance Battle Guide."

Part 8: Building Your Therapy Schedule

Age-Based Scheduling Guidelines

Age	Priority Therapies	Key Considerations
0–3 years	Early Intervention SLT, OT, developmental specialist, ABA	Home-based, family-centered; IFSP drives services
3–6 years	ABA (intensive), SLT, OT, school special ed	Peak brain plasticity — maximize hours; coordinate school + private
6–12 years	ABA (skill-focused), SLT, social skills group, OT for school	Balance therapy with school social inclusion; fatigue management
12–18 years	Social skills, vocational prep, SLT for pragmatics, OT for independence	Transition planning; self-advocacy skills; independence targets

Preventing Therapy Fatigue

- Schedule therapy at your child's highest energy times (typically morning for most children)
- Build in free, unstructured time every day
- Watch for signs of burnout: increased meltdowns, school refusal, loss of skills
- Quality of sessions matters more than total hours — a burned-out child doesn't learn

Master Therapy Tracker

Therapy	Provider	Days/Hours	Current Goals	Progress Rating
ABA				
Speech-Language				
Occupational Therapy				
Physical Therapy				
Social Skills Group				
Other:				

Resources

Resource	URL
BACB (Behavior Analyst Certification)	bacb.com
ASHA (Speech-Language Pathology)	asha.org
National Autism Center — Evidence Review	nationalautismcenter.org
PEERS Program	semel.ucla.edu/autism/peers

SpectrumReady

Empowering autism families with the knowledge they deserve.

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